



School based interventions for depression and anxiety in UK

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Mental health problems among adolescents, particularly anxiety and depression, are a growing public health concern (Marcheselli et al., 2018). In the UK, approximately one in four adolescents aged 17–19 meet the criteria for a diagnosable mental health disorder, and the impact of untreated mental health issues during this critical developmental stage can be profound (Newlove-Delgado et al., 2023). These issues are associated with a range of adverse outcomes, internationally (Johnson et al., 2018), including an increased risk of depression and anxiety in adulthood (Johnson et al., 2018; Kessler et al., 2005) as well as poorer academic performance (Brännlund et al., 2017), poorer education and employment outcomes (Hale et al., 2015) and challenges in forming and maintaining relationships (Sadler et al., 2018). Despite this, many young people struggle to access mental health support, with estimates suggesting that around 60% of young people with mental health issues do not receive any formal care (UK Government, 2022).

There are several barriers to solving these problems including the reluctance of young people themselves to seek help (Gulliver et al., 2010; Rickwood et al., 2005). Even when they do seek help, there is a low capacity in specialist mental health services for young people (McGorry et al., 2013) that results in very long waiting lists. So, despite knowing that the age of onset of most mental health problems is 15 for 50% of young people and 24 for 75% of young people (de Girolamo et al., 2012; Kessler et al., 2005), the barriers mean that early intervention is difficult.

School-based mental health interventions have been increasingly recognized as an essential component of early intervention strategies, providing a platform for reaching adolescents in an environment that is accessible, familiar, and less stigmatising than clinical settings (Fazel et al., 2014; Lovell & Richards, 2000). This has been recognised in the UK as well as in Low and Middle Income countries (LMIC) (Fazel et al., 2014).

Who should deliver the mental health interventions to reduce depression and anxiety in schools?

The attraction of using teachers for delivering mental health interventions is that they are already in schools,

and if trained, could offer effective and hopefully, cost-effective interventions. Unfortunately, this has not been found to be the case. Stallard et al. (2014) and others have found that clinicians were generally more effective than teachers (Fisak et al., 2011; Zhang et al., 2023), particularly in secondary schools. A recent UK development is the introduction of a new professional group of Mental Health Support Teams (MHSTs) to work directly in schools and provide early support for children and adolescents with mild to moderate mental health difficulties (Alderwick & Dixon, 2019). These MHSTs (consisting of master's or postgraduate diploma level therapists or junior therapists) aim to improve access to evidence-based interventions, including cognitive-behavioural therapy (CBT) (NHS England, 2023). With appropriate training, it is possible they could deliver clinically effective and cost-effective interventions.

What programmes should be offered?

Mental health literacy

It has often been assumed that mental health literacy, which refers to knowledge and intention to recognise symptoms and would lead to better help-seeking, but this has not always been found. Yamaguchi et al. (2024) found a teacher led 20 minute online mental health literacy programme for 15–16 year students in a small RCT ($n=270$) led to improvements in improved knowledge and recognition of necessity to seek help. However, little change was found on “intention to seek help”. Prabhu et al. (2024) reviewed studies about interventions of the mental health literacy of secondary school teachers, concluding this could be improved but the quality of the studies was variable. There have also been variations in effectiveness depending on who delivered the intervention.

Clinical programmes

School based programmes range from mindfulness to more CBT-like programmes developed in Australia (Friends, Resourceful adolescent Programme, Mood Gym) and USA (Penn Resilience Programme, Mindfulness) with only one in the UK for adolescents (DISCOVER).

Systematic reviews on the effectiveness of psychological interventions in school settings have discovered mixed findings on reductions of depression and anxiety (Caldwell et al., 2019; Feiss et al., 2019; Werner-Seidler et al., 2021). One reason may be the variation in the inclusion and exclusion criteria which has been highlighted in a recent meta-analysis (Zhang et al., 2023). There have been reports of variation in the effects of different types of interventions with Zhang et al. (2023) reporting that CBT programmes tended to be associated with higher effect sizes, a conclusion that was also supported in an anxiety focussed review (Fisak et al., 2011). However, in the first narrative analysis of school interventions, Caldwell et al. (2019) did not support this view as strongly. They concluded that CBT showed weak evidence of reducing anxiety but studies were small. And they found no evidence of differences in types of interventions being effective in reducing depression.

Recent trials in the UK have had mixed effects. One example of a study with nil effects is a recent large cluster RCT of over 8376 students in 85 schools where a mindfulness intervention was delivered by teachers (Kuyken et al., 2022). The proposed explanation for the lack of effects was low student engagement, indicated by student's home practice being low at both post-intervention and one-year follow-up.

However, another trial of 900 students in 57 schools in 4 regions in England (called BESST) demonstrated very positive results (Brown et al. 2024). We report on some of the factors that have surfaced in systematic reviews and our own studies that might account for the accessibility and efficacy of these interventions (Brown et al., 2022).

Different ways of offering school interventions

Within school settings, interventions are typically delivered through one of two main models: targeted interventions, which focus on students identified as being at high risk of mental health issues, and universal interventions, which are offered to all students, regardless of their mental health status. Universal interventions have the advantage of reducing stigma and promoting mental health awareness across the broader student population, but they can dilute the intensity of support provided to those in greatest need and have been shown to be less effective in reducing mental health issues for those who already exhibit problems (Montero-Marín et al., 2022; Stallard et al., 2012). Conversely, targeted interventions can be more resource-efficient by concentrating on students who are most at risk, but they may miss students who are struggling but do not meet the criteria for high-risk/elevated symptoms (Calear & Christensen, 2010; Radez et al., 2021) and can also increase feelings of stigma in the target group (Gronholm et al., 2018). Screening also requires extra resources.

The most effective way to offer these school-based interventions is currently unclear with one review suggesting that targeted interventions more effective (Werner-Seidler et al., 2021) but two others finding no differences (Fisak et al., 2011; Kapadia et al., 2022).

Self-referral methodology (and increased accessibility)

Self-referral is where individuals independently decide to participate by referring themselves, rather than relying on a professional. In our work, self-referral is one part of the 5 part model described in the evidence-based “PLACES” accessibility model that is designed to increase access by reducing stigma and increasing convenience of interventions (Brown et al., 2022). For example, the PLACES accessibility model uses everyday language (e.g. “stress”) instead of clinical terms (e.g. “depression” or “anxiety”) to reduce stigma. Another aspect is that interventions are delivered in non-stigmatising settings that are convenient to the target population. Self-referral systems, have seldom been employed in trials of school based interventions (Brown et al., 2010) despite offering benefits such as lowering stigma (Brown et al., 2010) and also reinforcing autonomy that is particularly valued by adolescents (Wilson & Deane, 2012). They also allow for more efficient resource use with high levels of engagement from students who may not have previously sought help, better follow-up rates (Brown et al., 2019), and greater participation from minoritised groups that are frequently under-served (Brown et al., 2019).

Co-design

Co-design where students help design the intervention is growing in importance. It is an important issue given that professionals' and users' perspectives about what are important aspects may vary (Wittevrongel et al., 2025).

The DISCOVER workshop programme was co-designed with 16–18 year olds (Sclare et al., 2015) and is designed to enhance student-clinician interaction and increases the use of visual aids, including videos and group exercises. Co-design was achieved through extensive consultation with young people who provided feedback on the intervention workshop's format, content, and delivery style, ensuring the program addressed the specific needs and preferences of this age group. Their input shaped various aspects of the intervention, including the program name as well as refining materials and interactive tasks.

However, possibly because of the newness of this approach, much of the literature is devoted to describing different methods of co-design and evidence about the effectiveness of co-design of mental health intervention about greater engagement is still very limited (Slattery et al., 2020). Nevertheless, qualitative studies have been promising. A review of PPI work with young people

has also concluded that more operationalisation and consistency of evaluation measures was needed (Totzeck et al., 2024). For example, the effects of the participation on participants and whether their input changed the research process or researchers' opinions could be noted.

The BESST trial – incorporating knowledge so far

The BESST trial (Brown et al. 2024) incorporated several elements:

- a co-designed intervention with 16–18 year olds (Sclare et al. 2015)
- the non-diagnostic self-referral process, described in the PLACES model (Brown et al., 2022)
- a brief CBT programme for 16–18 year olds
- delivered by mental health support teams in schools

The self-referral pathway (Brown et al., 2022) was offered at school assemblies when students were invited to opt-in into the trial and stress workshops. Stress problems were described in ordinary ways such as school exams and family pressures. The DISCOVER intervention was an one-day, CBT-based workshop which was designed to help adolescents manage stress, anxiety, and low mood by providing practical strategies for coping with mental health challenges (Sclare et al., 2015).

This led to 900 students referring themselves to the trial with 46% of these students being from minority groups. The workshop intervention was each run by 3 MHST clinical staff in groups of up to 19 students in 4 English regions. The trial provided robust evidence for the clinical ($ES = -0.17$) and cost-effectiveness of a school-based intervention for adolescents especially for those with elevated depressive symptoms ($ES = -0.52$) (Brown et al., 2024). Process evaluation results were positive among students themselves and staff delivering the workshops (Weaver et al, in revision). The results are consistent with Kuyken et al.'s (2023) reflections in 2023 after his unsuccessful trial about the need for good reach, student engagement, co-design of the intervention and the need to think about sustainability of the intervention through the MHSTs.

The future for school-based interventions

Andrews and Foulkes (2025) have also strongly recommended that that we need to move away from universal interventions for students. They recommend more opt-in interventions which are more in line with adolescents' desire for autonomy, which is equivalent to the self-referral system. The BESST trial seems to show that these factors do make for better accessibility and a more effective and cost-effective approach.

We make two recommendations. One is that this field should assess actual engagement of students as opposed to attitudes to help-seeking. While mental health literacy and the relationship with attitudes to help-seeking have been frequently studied, the effects on actual help-seeking have not been shown. The PLACES model is probably one of the few models that does show actual help-seeking behaviour (Brown et al., 2022) improves among young people as well as adults with different problems. Secondly, we recommend that the DISCOVER workshop, and the PLACES accessibility model form the basis for offering school interventions to 16–18-year-olds in schools. We are not claiming it is perfect and we are researching how it might be implemented across England as understanding the context of school interventions is very important (Roshan et al., 2025). However, it does appear very effective, clinically and cost-effectively, and also engages diverse students very well, and all this at a time when other interventions have been disappointing. We hope this may help start to reduce the rising tide of mental health problems among young people.

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